

Dr. Nejat,

Thank you for your interest in the MiniBox+. Since you already do spirometry, I want to make the case for the two tests that turn a spirometer into a full pulmonary function lab, lung volumes and DLCO, and then show you exactly how to document them so they get paid in New York. I have put a reference behind every clinical and billing claim below so you can verify it yourself. The links are at the end.

What spirometry alone tells you, and what it misses. Spirometry (FEV1, FVC, FEV1/FVC, reversibility) is the workhorse of asthma, and current GINA guidance is built around it: spirometry before and after bronchodilator at diagnosis, 3 to 6 months after starting ICS, then periodically [1]. But spirometry only sees the airways, not what is behind them. Two patients with the same FEV1 can have very different lungs.

Lung volumes (TLC, FRC, RV), what spirometry cannot see.

- **Restriction vs. pseudo restriction.** A low FVC on spirometry looks restrictive, but the 2022 ERS/ATS interpretation standard is explicit: a low FVC alone does not establish restriction. Only a measured TLC can confirm or rule it out [2]. Without lung volumes you risk mislabeling an asthmatic with air trapping as restrictive, or missing true restriction.
- **Air trapping and hyperinflation.** An elevated RV and RV/TLC quantify the air trapping behind poorly controlled or chronic asthma. That is an objective number you can trend and use to justify stepping up therapy [2].

DLCO, the test that separates asthma from everything else.

- DLCO is typically normal (or high) in pure asthma, and reduced in emphysema or COPD overlap because of lost gas exchange surface. It is the single most useful number for telling airway disease apart from parenchymal disease [3][4].
- That makes it the test that catches the asthmatic who is not responding, the smoker with asthma and COPD overlap, and the interstitial or eosinophilic processes on your differential. These are patterns spirometry looks right through.
- For biologic candidates (omalizumab, dupilumab, mepolizumab, benralizumab, tezepelumab), a baseline full PFT documents phenotype and supports the therapy decision.

The bottom line: spirometry answers “is there obstruction, and does it reverse?” Lung volumes plus DLCO answer “is it really asthma, how severe is the air trapping, and is something else going on?” The MiniBox+ runs all three, spirometry, full lung volumes, and DLCO, on a desktop, no booth or gas plumbing, in roughly 15 to 20 minutes, run by your existing staff. Its lung volume accuracy was validated in a 266 patient multicenter study published in CHEST [5].

How it bills

Same CPT codes as a traditional PFT lab:

CPT	Test	Medicare (approx.)
94010	Spirometry	~\$27
94060	Spirometry pre/post bronchodilator (reversibility, includes 94010)	~\$38
94726	Lung volumes, plethysmography (TLC, FRC, RV)	~\$55
94729	DLCO, diffusing capacity (add on)	~\$54
Full panel	94060 + 94726 + 94729	~\$147

Check your own rates: commercial plans typically pay more than Medicare. Confirm your New York locality on the CMS Physician Fee Schedule Lookup Tool [6]. Also available as indicated: 94070 (methacholine or bronchial challenge), 95070 (inhalation challenge), 95012 (FeNO).

Getting it paid in New York, what to put in the chart

New York's Medicare contractor is National Government Services (NGS, Jurisdiction K), and its Pulmonary Function Testing policy is the rulebook [7][8]. The single most important rule: **Medicare does not cover screening of an asymptomatic patient.** Every test needs a documented symptom or diagnosis and a physician order.

1. You do not need a confirmed asthma diagnosis to test. A symptom is enough. Code the presenting symptom as primary:

- Shortness of breath or dyspnea, R06.00
- Wheezing, R06.2
- Cough (chronic or unspecified), R05.3 / R05.9
- Chest tightness, exercise intolerance, and similar

Add the patient's known allergic condition (allergic rhinitis, atopy) as secondary to show why airway disease is the concern. One sentence of rationale in the note, for example "Dyspnea and wheeze in a patient with allergic rhinitis. Full PFT to characterize obstruction, air trapping, and gas exchange," is what carries the claim.

2. For confirmed asthma, code the type and severity: J45.x (for example J45.40 / J45.41 moderate persistent, J45.50 / J45.51 severe persistent, J45.901 / J45.902 with exacerbation).

3. How often can he test? There is no fixed numeric cap. Medicare pays as often as it is medically necessary and documented. What is not allowed is routine, repetitive batteries of tests without a reason [8]. In practice a full panel is supportable at diagnosis, at each meaningful change in symptoms or control, before and after a therapy change, and to confirm response. So

a patient whose asthma is changing can legitimately be tested several times a year.
(Commercial plans often want prior authorization above roughly 2 to 3 panels per year.)

Billing rules, verified:

- Do not bill 94010 and 94060 on the same date. 94060 already includes the baseline spirometry [8].
- 94729 (DLCO) is an add on. Bill it with spirometry or lung volumes, never alone [8].
- 94726 and 94729 are billable in the same session. That is the complete panel combination.
- If a separately identifiable office visit happens the same day, append modifier 25 to the E/M code (for example 99213 with modifier 25), not to the PFT.
- Keep on file: the physician order with clinical indication, an acceptable tracing (spirometry needs at least 3 acceptable efforts), and a signed interpretation by the ordering physician [7][8].

What it costs to run, per patient

- Mouthpiece: about \$3.16 each, and bulk pricing brings that down further.
- DLCO gas: typically \$1.50 to \$3.00 per patient.
- Annual maintenance: none.

That puts you at roughly \$5 per patient in total cost to run a full panel, before staff time. Set that against a full panel that reimburses around \$147 and the margin on every test is clear.

The investment

I do not want to bury this in detail. The all in price came in at \$37,800, below the figure we discussed when I was in your office. The quote (number 00007445) is attached and includes the MiniBox+, the cart, a 4 year extended warranty, DLCO gas regulators, and a starter set of 100 tests with filters and clips.

This should give you and your team a good start to verify the clinical case, the billing, and the numbers on your end. If anything else would help, just tell me what you need and I will get it to you. And if questions come up as you review it, I am happy to get on a quick call with whoever needs to weigh in.

Best regards,

Erik Bernard

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References, check them yourself

1. GINA 2024 Global Strategy for Asthma Management and Prevention: ginasthma.org/2024-report
2. Stanojevic S, et al. ERS/ATS technical standard on interpretive strategies for routine lung function tests. Eur Respir J 2022;60:2101499: publications.ersnet.org (PubMed: [pubmed 34949706](https://pubmed.ncbi.nlm.nih.gov/34949706/))
3. Diffusing Capacity of the Lungs for Carbon Monoxide (DLCO), StatPearls / NCBI: ncbi.nlm.nih.gov/books/NBK556149
4. Diffusing Capacity, the Too Often Ignored Lung Function Test. CHEST 2021: journal.chestnet.org
5. Validation of a Novel Compact System for the Measurement of Lung Volumes (MiniBox), CHEST 2021 (Berger KI, et al.): [pmc PMC8411450](https://pubmed.ncbi.nlm.nih.gov/34949706/)
6. CMS Physician Fee Schedule Lookup Tool: cms.gov/medicare/physician-fee-schedule/search
7. National Government Services (NGS), Medicare Part B MAC, Jurisdiction K (includes New York): ngsmedicare.com
8. CMS Billing and Coding: Pulmonary Function Testing (Article A57216): [cms.gov MCD A57216](https://cms.gov/MCD-A57216)

Attachments

- Allergy_Will_You_Get_Paid_Doctor_Reference.pdf, the allergy and asthma CPT, coverage, frequency, and ICD-10 pairing sheet.
- CHEST 2021 validation study (reference 5).
- MiniBox+ product spec or one pager.
- Dr_Nejat_Quote_00007445.pdf, the formal quote (\$37,800).
- Dr_Nejat_ROI_Allergy, the return on investment summary.